

Breast Reconstruction Consultation: Assessment and Plan Template

Version 1.5, September 18, 2026. Built from the review of nine colleague notes across three providers, the ABPS 2026-2027 Booklet of Information requirements (re-read in full against this template on September 17, 2026; the coverage map is docs/B0I-Coverage-Map.md), the examiner rating items, the user's own dictated sentences, and a fourth colleague's procedure-explanation templates (Provider D, reviewed September 18; see docs/Note-Review-Log.md). Every paragraph is written in the user's voice. Brackets are dictated fields; the scribe leaves them blank if not stated so gaps are visible at sign-off. Delete any module that does not apply; never leave education or consent text for a modality the patient is not receiving.

Part 1. Style rules (apply to anything added later)

1. First person, past tense, for what you did: "I reviewed," "I explained," "I recommended."
2. The patient's response is its own sentence with the patient as subject: "Patient verbalized understanding that..." "Patient wishes to proceed."
3. Conditions carry an explicit then: "If [], then [], and in that case []."
4. Paragraphs open with a sequence marker where one fits: "After discussion of...", "At the conclusion of the consultation..."
5. Sentences close with purpose or contrast where it adds meaning: "in order to...", "rather than risk...", "as well as..."
6. Use the recurring nouns: surgical plan, surgical option, reconstructive goals, her concerns, a later stage, revision stage.
7. Plain sentences of 20 to 35 words, one idea each, in prose; the only list is inside the risk sentence.
8. No sentence may contradict the history, the exam, or another sentence in the plan.

Part 2. Fixed practice lines

- Bilateral DIEP: 6 to 8 hours under general anesthesia, at least 2 inpatient nights, 6 to 8 weeks to full activity.
- Expander or implant reconstruction: 3 to 4 hours under general anesthesia, 23-hour observation, 4 to 6 weeks to full activity.
- Expander plane is an intraoperative decision (paragraph in Part 4, module E).
- For a grade III breast reconstructed with a flap, the skin pattern (Wise-pattern skin-reducing mastectomy versus skin-sparing mastectomy with a later mastopexy) is discussed with the breast surgeon before the surgical date is set (module M3).
- Risks are listed in every note where a surgical plan is offered.
- A1C is ordered only for diabetic patients.
- Every stage is at least 3 months apart; the definitive stage after radiation is at least 6 months from the completion of radiation.
- Every stage is its own case on the Case Log: its own in-person preoperative visit at least the day before that surgery, its own operative consent, its own note with a risk sentence, its own preoperative, intraoperative, and 90-day photographs. At most three cases per patient count toward the 50.
- The breast surgeon is a non-plastic-surgery co-surgeon: the case is entered, not flagged as a co-surgeon case, with my own operative report, my own bill, and only the plastic surgery portion's skin-to-skin time.
- Any overnight stay, including 23-hour observation, is entered as inpatient on the Case Log.

- Every consultant whose report shaped the plan is named with the date of the report, because their reports go into the Initial Evaluation tab of the case report.

Part 3. Assessment (numbered, declarative)

1. [Age]-year-old [pre/postmenopausal] woman with [side] breast [histology], [grade], cT[] ([] cm at [clock position], [] cm from the nipple) cN[], ER/PR [], HER2 [], genetics [negative/pending/not indicated], family history []. [Second lesion if multicentric.] Planned [unilateral/bilateral] [nipple-sparing/skin-sparing/skin-reducing] mastectomy with [sentinel node biopsy/axillary dissection] by Dr. [] on [date]. [Contralateral mastectomy is risk-reducing by informed preference.]
2. Adjuvant therapy anticipated: [chemotherapy: none / neoadjuvant, last cycle (date), surgery 4 to 8 weeks after / adjuvant, to start within 30 to 60 days of surgery / pending Oncotype]; radiation [unlikely / possible / planned], based on [nodes, T stage, margins, response]; endocrine or targeted therapy []. Systemic decisions are per Dr. [medical oncology]; radiation per Dr. [radiation oncology].
3. Reconstruction candidacy: patient prefers [own tissue / implants / undecided] and a [same / smaller / larger] size. [Donor site: abdominal pinch () cm above and () cm below the umbilicus, scars (), estimated flap volume () mL per side against breast volume about () mL / abdomen unavailable because (), thigh and gluteal tissue ().] [Nipple-sparing: anatomically (favorable/borderline/not a candidate) with SN-N () cm, N-IMF () cm, ptosis grade (); oncologic eligibility per Dr. ().]
4. Perioperative risk: ASA [], Caprini [number], nicotine [never / quit (date) / current], BMI [], [diabetes with HbA1c () / no diabetes], anticoagulation [], hormone therapy [], medications [], allergies [], [MRSA history / prior radiation / thrombotic history].
5. Records reviewed at this visit: pathology dated [], [mammogram / ultrasound / MRI] dated [], consult notes from Dr. [breast surgeon] dated [], Dr. [medical oncology] dated [], [Dr. (radiation oncology) dated ()], [genetics dated ()], [tumor board dated ()]. [Clinical trial enrollment: none / enrolled in () with protocol consent in the chart.]

Part 4. Plan (narrative, in order)

A. OPTIONS (EVERY NOTE)

I reviewed the options with the patient, which included no surgical reconstruction with a flat closure or external prosthesis, delayed reconstruction after cancer treatment, implant-based reconstruction done in a one or two stage manner, and autologous reconstruction as well. For each surgical option I explained how it could or could not address her concerns and reconstructive goals. [One sentence per option that was set aside and why: "Abdominal flap reconstruction is not possible because of her prior abdominoplasty." "Immediate implant reconstruction is not recommended because radiation is expected." "She is not interested in a flat closure or a prosthesis."]

B. RECOMMENDATION (EVERY NOTE)

After discussion of the surgical options, I recommended [procedure, laterality, staging], because [the two or three facts from the assessment that decide it: measurements, oncologic timing, radiation likelihood, donor site, patient goals]. Patient [agrees with this plan / elected () after discussion of ()].

C. RISKS (EVERY NOTE)

After discussion of the surgical options, I explained the risks of the surgery, specifically stating the risks of [insert the list from Part 5 for the chosen pathway, plus the patient-specific items], as well as the alternatives to the surgical plan and the expected benefits. I explained that exact symmetry is rare and difficult to achieve surgically, and that no

promise of exact symmetry has been made nor should be expected. I explained how each of these risks is reduced, specifically [ICG angiography of the skin flaps, perioperative antibiotics, sequential compression devices and enoxaparin, drains, nicotine abstinence, glycemic control], and how each would be treated if it occurred, including [return to the operating room, removal of the device, debridement, or delayed reconstruction]. [I provided written information about ().] Patient verbalized understanding.

D. BACKUP PLAN (EVERY NOTE)

I explained that if [the mastectomy skin flaps are poorly perfused on ICG angiography], then [the expander will be left deflated or reconstruction delayed], and that if [], then []. [For flaps: "If the CT angiogram does not show adequate perforators, then the plan will change to a muscle-sparing TRAM or a thigh flap. If the flap is not viable during surgery, then a tissue expander will be placed as a staged alternative."] Patient verbalized understanding that the plan may change in the operating room in order to protect the skin envelope and the reconstruction.

E. EXPANDER PLANE (IMPLANT-BASED NOTES)

I explained that the expander plane will be decided in the operating room after the mastectomy. This decision is based on the thickness and quality of the mastectomy skin flaps on direct inspection, the perfusion of the skin flaps and nipples on ICG angiography, the amount of any poorly perfused skin that may need to be excised, the integrity of the pectoralis major after the mastectomy, the size and ptosis of the breast and the expected final implant volume, whether radiation is anticipated, and her own risk factors including BMI, nicotine exposure, diabetes, and prior radiation. I explained that well-perfused flaps of adequate thickness favor prepectoral placement with mesh in order to avoid animation deformity, and that thin or marginally perfused flaps favor subpectoral placement for muscle coverage, a lower initial fill, or a deflated expander. Patient verbalized understanding that either plane may be used.

F. RADIATION CONTINGENCY (EVERY NOTE WHERE RADIATION IS POSSIBLE)

Expander version. I explained that if the final pathology of the specimen demonstrates a positive node, then radiation becomes likely, and in that case the expander will stay in place through radiation and be exchanged at a later stage, at least 6 months from the completion of radiation. Patient verbalized understanding that the need for radiation can change her expander fill and timing, and that conversion to her own tissue at that stage would be discussed.

Flap version. I explained that if the final pathology of the specimen demonstrates a positive node, then radiation becomes likely, and in that case the flap will be radiated, which can cause firmness, shrinkage, and fat necrosis, and any revision will wait at least 6 months from the completion of radiation. Patient verbalized understanding.

Radiation already planned. I explained that radiation is planned after her mastectomy, and that in that case [the expander will be filled in the operating room to the volume agreed with Dr. (radiation oncology) with no fills after simulation / the reconstruction will be delayed until at least 6 months from the completion of radiation]. I explained that implant reconstruction after radiation carries a much higher risk of infection, capsular contracture, thinning of the skin, exposure, and removal, and that reconstruction with her own tissue tolerates radiation far better. Patient verbalized understanding.

G. STAGING AND TIMELINE (EVERY NOTE)

I explained that the reconstruction will involve [number] stages, each at least 3 months apart, with [exchange at about 3 months if no adjuvant treatment / exchange or conversion at least 6 months from the completion of radiation / revisions, nipple reconstruction, and contralateral symmetry at 3 months or later]. [If chemotherapy is planned after surgery: "I explained that chemotherapy is expected to start within 30 to 60 days of surgery, and that the first priority of the reconstruction is a healed wound so that treatment is not delayed."] The index operation is expected to take [hours] under general anesthesia with [23-hour observation / at least 2 inpatient nights] and a recovery of [4 to 6 / 6 to 8] weeks.

H. SAFETY AND COORDINATION (EVERY NOTE)

The surgery will be performed at [facility, accredited] under general anesthesia. Her Caprini score is [number], and she will receive sequential compression devices [and enoxaparin for () days]. [Nicotine, glycemic, anticoagulation, or MRSA line as applicable.] [She is enrolled in the () clinical trial; the protocol consent and IRB approval are in the chart.] I will coordinate with Dr. [breast surgeon] regarding [incision, skin pattern, nipple decision, axilla], with Dr. [medical oncology] regarding [timing of systemic therapy], [with Dr. (radiation oncology) regarding volume and timing], [and with genetics regarding the pending result].

I. ORDERS, EACH WITH ITS INDICATION (EVERY NOTE)

Routine labs [with PT and PTT because microsurgery is planned], chest radiograph, ECG [per anesthesia], [A1C because she is diabetic], [CT angiogram abdomen and pelvis with 0.75 to 1 mm cuts, creatinine first, to map the perforators], [echocardiogram per oncology], [MRSA screen because of her history], [anticoagulation clearance letter from Dr. ()], [genetic testing because of ()], [mammogram because she is 40 or older and none is on file]. Nothing is ordered without an indication that a reader can find in this note.

J. FOLLOW-UP (EVERY NOTE)

I will see the patient in person for a preoperative visit at least one day before surgery on [date] in order to examine her again, sign consent, and mark. I will see her myself immediately after surgery and in person within 30 days of surgery on [date], [at the start and midpoint of radiation,] and at 90 days after surgery on [date], when I will take her postoperative photographs; visits in between may be by telemedicine. [Each later stage will have its own in-person preoperative visit at least the day before that surgery.] [For radiated patients: "She will return at 6 months from the completion of radiation to schedule the definitive stage."]

K. CLOSING (EVERY NOTE)

At the conclusion of the consultation, the patient verbalized her understanding of the plan back to me and stated that all of her questions had been answered. Patient wishes to proceed. Preoperative photographs were taken with the patient's consent, [no modeling software was used / images generated with () were reviewed with her and saved to the chart], and I explained that photographs will also be taken during surgery and at each visit as part of her surgical record. The patient signed and dated the records and photograph consent for examination and certification purposes by The American Board of Plastic Surgery, which was witnessed, together with the practice's own consent. We will obtain insurance authorization; coverage of all stages, contralateral symmetry, and prostheses under WHCRA was explained.

Part 5. Risk lists (insert into paragraph C)

General, every plan: bleeding, hematoma, seroma, infection, delayed wound healing and dehiscence, mastectomy skin flap necrosis, unfavorable scarring, asymmetry, contour irregularity, chronic pain, numbness of the chest and reconstructed breast, deep venous thrombosis and pulmonary embolism, anesthetic complications, need for revision or additional surgery, need for contralateral procedures, and dissatisfaction with the final result.

Implant-based, add: capsular contracture, implant rupture or leakage, malposition or rotation, rippling and palpability with prepectoral placement, animation deformity with subpectoral placement, implant exposure and removal, breast implant-associated anaplastic large cell lymphoma and squamous cell carcinoma, systemic symptoms reported with implants, the fact that implants are not lifetime devices and will likely need replacement, the FDA recommendation for MRI or ultrasound screening of silicone implants beginning 5 to 6 years after placement and every 2 to 3 years thereafter, and, for acellular dermal matrix, allergic or immune reaction, seroma, failure to integrate, and infection. [Add: "I reviewed the FDA boxed warning and the Patient Decision Checklist, which will be signed at the preoperative visit."]

Autologous, add: thrombosis of the microvascular connections requiring an emergent return to the operating room for flap salvage, partial or total flap loss with removal of the flap, fat necrosis presenting as a firm area that is distinguished from recurrence by imaging, abdominal bulge or hernia, abdominal weakness, need for mesh reinforcement of the abdominal wall, loss of the umbilicus with reconstruction at a later stage, donor-site seroma and delayed healing, need for a vein graft or alternate recipient vessels, injury to adjacent vessels, nerves, muscles, and the viscera of the chest and abdomen, transfusion, changes in sensation of the chest and abdomen, contour irregularities at the breast and the donor site, longer hospital stay and recovery, and the fact that the abdomen can be used only once.

Nipple-sparing, add: partial or complete nipple necrosis, loss of nipple and areolar pigmentation, nipple malposition, temporary or permanent loss of nipple sensation, inability to breastfeed, and the small possibility of cancer in retained breast tissue behind the nipple, which requires continued surveillance of the nipple-areola complex.

Oncoplastic reduction with lumpectomy, add: persistent symptoms; positive margins requiring re-excision or completion mastectomy, which is harder after tissue rearrangement; fat necrosis and microcalcifications that can complicate future imaging and biopsy; partial or complete nipple necrosis and the need for a free nipple graft with loss of sensation and pigmentation; breakdown at the T-junction that could delay radiation; radiation-related shrinkage, firmness, and asymmetry of the treated breast; areolar wrinkling or deformity; recurrent hypertrophy with weight gain and change in appearance with weight change or pregnancy; an unexpected cancer in the contralateral reduction specimen.

Patient-specific, add as applicable: higher infection risk with MRSA history or obesity; higher implant failure with obesity or radiation; thrombotic risk with interruption of anticoagulation; higher flap risk with a thrombotic history; delay of adjuvant chemotherapy if a wound complication occurs; T-junction breakdown with a skin-reducing pattern.

Part 6. Modules (insert where they apply)

M1. BORDERLINE NIPPLE-SPARING

The patient's anatomy raises my concern for nipple preservation, specifically [SN-N () cm, N-IMF () cm, ptosis grade (), prior periareolar scars]. I explained that due to the anatomy the risk of losing part or all of the nipple is higher than usual. I explained that due to positioning her nipples will sit low after reconstruction and will likely need to be repositioned at a later revision stage in order to correct their position. I also explained that if the nipples look poorly perfused during surgery, then I will either convert them to a free nipple graft or remove them rather than risk a wound problem. Patient verbalized understanding and wishes to attempt nipple preservation with these risks in mind. Oncologic eligibility for nipple preservation rests with Dr. [breast surgeon].

M2. NIPPLE NOT A CANDIDATE

I explained that the nipple cannot be preserved because [tumor involvement / retroareolar location / positive margins / SN-N () cm with grade III ptosis], and that the nipple will be removed with the specimen and reconstructed or tattooed at a later stage. [For pendulous breasts: "I explained that a free nipple graft onto the flap is an alternative, with the expected loss of sensation, projection, and pigment, and the patient (elected / declined) this option."] Patient verbalized understanding.

M3. GRADE III BREAST WITH A FLAP: SKIN PLAN

I explained that because of her degree of ptosis the skin envelope requires a plan of its own. I explained that there are two approaches: a Wise-pattern skin-reducing mastectomy in which the flap skin fills the lower pole, which lifts the breast at the first operation but carries a risk of breakdown at the junction of the incisions, or a skin-sparing mastectomy in which the envelope is left loose and lifted at a later revision stage once the blood supply has recovered.

I explained that I will discuss the skin pattern with Dr. [breast surgeon] and that the plan will be finalized with the patient at the preoperative visit. Patient verbalized understanding that a second stage is part of the plan rather than a complication.

M4. DELAYED-IMMEDIATE EXPANDER WHEN RADIATION IS LIKELY

I recommended a tissue expander at the time of mastectomy in order to preserve the skin envelope while radiation is completed, with the definitive decision between an implant and her own tissue made at least 6 months from the completion of radiation. I explained that an implant radiated in this setting fails in roughly one of five patients and develops significant capsular contracture in a large minority, and that conversion to her own tissue at that stage avoids most of that risk. Patient verbalized understanding and prefers this approach to [a flat closure with delayed reconstruction / an immediate flap that would be radiated].

M5. DELAYED RECONSTRUCTION AFTER COMPLETED RADIATION

I explained that the best window for reconstruction with her own tissue is 6 to 12 months from the completion of radiation, once the skin has softened and the acute changes have resolved, and that complications are highest between 3 and 6 months after radiation. I explained that an expander in radiated skin carries a higher risk of failure and would likely require a latissimus flap or fat grafting to help the skin. Patient verbalized understanding and elected [].

M6. CONTRALATERAL PROPHYLACTIC MASTECTOMY

I explained that removal of the contralateral breast does not improve survival from her current cancer, that it removes the need for future surveillance of that breast and can simplify symmetry, and that the decision is hers in consultation with Dr. [breast surgeon]. [If genetics pending: "I explained that a positive genetic result would change this discussion and would add counseling regarding her ovaries."] Patient elected [].

M7. PENDING RESULTS GATE THE DATE

I explained that the surgical date will be set after [outside records, the MRI, genetic testing, the medical oncology consultation] are reviewed, because [the extent of disease decides nipple preservation / the subtype decides whether chemotherapy comes before surgery / the genetic result changes the contralateral discussion]. Patient verbalized understanding.

M8. DCIS CONTINGENCY

I explained that in about one of five patients with DCIS the final pathology shows invasive cancer, that a sentinel node biopsy will be performed at the mastectomy because it cannot be performed afterward, and that if invasive disease or a positive node is found, then chemotherapy or radiation may become part of her treatment and the reconstruction will be sequenced around it. Patient verbalized understanding.

M9. SUBTYPE-DRIVEN SEQUENCING (TRIPLE NEGATIVE OR HER2-POSITIVE)

I explained that for her cancer type chemotherapy [with pembrolizumab / with trastuzumab and pertuzumab] is usually given before surgery, that the mastectomy and reconstruction are planned 4 to 8 weeks after the last cycle once her counts have recovered, and that the final pathology after chemotherapy will determine whether radiation is needed. [For surgery first: "I explained that chemotherapy is expected to start within 30 to 60 days of surgery, and that a wound complication could delay it, which is a risk of immediate reconstruction that she accepts."] Patient verbalized understanding. Systemic therapy decisions are per Dr. [medical oncology].

M10. PATIENT DECLINING RECOMMENDED SYSTEMIC THERAPY

The patient stated that she would like to avoid chemotherapy even if it is recommended. I explained that this decision belongs to her and her medical oncologist, that I encouraged her to keep that consultation, and that the reconstruction plan does not depend on her decision. Patient verbalized understanding.

M11. BRIDGING SENTENCE WHEN PREFERENCE AND PLAN DIFFER

The patient initially asked about [implant reconstruction]. After discussion of [her BMI and implant failure risk / her wish to keep her current size / the expected radiation / the durability of her own tissue], she elected [autologous reconstruction] in order to []. Patient verbalized understanding of the longer recovery and the abdominal scar.

M12. UNILATERAL RECONSTRUCTION AGAINST A NATIVE BREAST

I explained that matching a single reconstructed breast to her native breast is more difficult than a bilateral reconstruction, that her own tissue matches a native breast better over time than an implant, and that a contralateral [reduction / lift / augmentation] at a later stage is likely to be needed for symmetry and is covered under WHCRA. Patient verbalized understanding.

M13. COMORBIDITY LINES

Obesity. I explained that her BMI of [] raises the risk of wound complications with either approach and raises the risk of implant failure specifically, which favors her own tissue if she is otherwise a candidate. *Nicotine.* I explained that she must be free of nicotine in all forms for at least 4 weeks before and 4 weeks after surgery, and that I may test for this before surgery. *Diabetes.* I explained that her hemoglobin A1C must be below 8 before elective surgery, and that she will be referred to her primary physician if it is not. *Anticoagulation.* I explained that her anticoagulation will be held and restarted per written instructions from Dr. [], and that her thrombotic history raises the risk of flap thrombosis, which is why []. *MRSA.* I explained that her history of MRSA raises the risk of implant infection, that she will complete a decolonization protocol before surgery and receive appropriate antibiotics, and that I will have a low threshold to remove the device if infection develops. *Hormone therapy.* I explained that [estrogen-containing medication will be stopped 4 weeks before surgery / tamoxifen may be held for 28 days before a free flap as a decision shared with her oncologist].

M14. OPENING AND THE NATURE OF RECONSTRUCTION (EVERY NOTE)

I began the consultation by reviewing her diagnosis of [], the anticipated [mastectomy and adjuvant plan], and her goals for reconstruction, in the context of her medical and surgical history. I explained that breast reconstruction is an elective operation, and that its goal is to replace the volume removed at mastectomy with either a device (a tissue expander or implant) or her own living tissue (a flap). For each approach I reviewed the initial operation, the surgical sites and scars, the secondary operations, the maintenance the reconstruction requires over time, the hospital stay, the recovery, the benefits, and the short- and long-term complications.

M15. MASTECTOMY AND NIPPLE COUNSELING (EVERY MASTECTOMY NOTE; CONSENT FOR THE MASTECTOMY ITSELF RESTS WITH THE BREAST SURGEON)

I explained that the mastectomy removes all tissue that can be identified as breast, that some breast cells inevitably remain, and that the operation reduces but does not eliminate her future risk. [For nipple preservation:] I explained that a nipple-sparing mastectomy leaves a small amount of ductal tissue behind the nipple, that this has been studied and is an oncologically valid option in appropriate patients, and that oncologic eligibility rests with Dr. [breast surgeon]. I explained the alternative of a skin-sparing mastectomy with removal of the nipple-areola complex and later nipple reconstruction or three-dimensional tattooing, and that a nipple-sparing mastectomy is performed through an inframammary fold incision whereas a skin-sparing mastectomy leaves an oblique or transverse scar across the breast. I explained that nipple preservation carries a higher risk of skin loss around the areola and of partial or complete nipple

loss, loss of pigmentation, and temporary or permanent change in sensation, and that it requires continued surveillance of the nipple-areola complex. Patient verbalized understanding and [wishes to preserve her nipples with these risks in mind / elected a skin-sparing mastectomy].

M16. DIEP FLAP: HOW THE OPERATION IS DONE (AUTOLOGOUS NOTES)

I explained how the reconstruction is performed. An elliptical segment of skin and fat is taken from the lower abdomen between the umbilicus and the pubis and isolated on its perforating blood vessels. The rectus fascia is opened and the vessels are followed through the muscle to the deep inferior epigastric vessels where they arise in the pelvis. I explained that every effort is made to preserve the muscle, the fascia, and the motor nerves, but that if the vessels are small or delicate, then a cuff of muscle and fascia is taken with the flap as a muscle-sparing TRAM, and in that case the abdominal wall carries a higher risk of bulge, hernia, and weakness and may need mesh reinforcement. I explained that a CT angiogram is obtained before surgery in order to confirm her candidacy and plan the perforators [in relation to her prior abdominal scars]. I explained that the flap is brought to the chest and its artery and vein are connected under the microscope to the internal mammary vessels [with removal of a small segment of rib cartilage if needed], that these connections are delicate and can clot, that a clot requires an emergent return to the operating room to attempt salvage, and that although this is rare it can result in total loss of the flap and of the reconstruction. I explained that for this reason she will stay in the hospital for at least 2 nights with intensive monitoring of the flap's blood flow. I explained the inset and shaping of the flap, the scar pattern on the breast, [that a small skin island is kept for monitoring and removed at the revision stage,] and that without clothing it will be apparent that she has been operated on. I explained that the abdomen is closed with a low transverse scar and a scar around the umbilicus, that this closure shares principles with an abdominoplasty but is not the same operation, and that in select cases the umbilicus cannot be preserved and would be reconstructed at the revision stage. I reviewed the perioperative expectations, hospital course, recovery, restrictions, time off work, and the stages involved in reaching the final result. Patient verbalized understanding.

M17. EXPANDER OR DIRECT IMPLANT: HOW THE OPERATION IS DONE (IMPLANT-BASED NOTES)

I explained the difference between a tissue expander, which is a two-stage reconstruction, and a direct implant, which is a one-stage reconstruction, and that the choice between them is made in the operating room based on the quality of the mastectomy skin flaps and the pocket, judged by direct inspection and ICG angiography. I explained that in rare cases, if there is significant concern about healing, then immediate reconstruction is abandoned and reconstruction is pursued in the delayed setting. I explained that if an expander is placed, then she will return to the office for serial expansions until the desired volume is reached, and that a second outpatient operation exchanges the expander for the permanent implant. We discussed saline and silicone implants and [she chose silicone]. I explained that acellular dermal matrix may be used to support and position the device and may reduce the risk of capsular contracture, that this is an off-label use of an FDA-approved product that is common in breast reconstruction, and that it carries its own risks of allergic or immune reaction, seroma, failure to integrate, and infection. I explained that secondary procedures to improve shape and symmetry are common and may include adjustment of the implant pocket, fat grafting, implant exchange, and scar revision, each at least 3 months apart. Patient verbalized understanding.

M18. ONCOPLASTIC REDUCTION WITH LUMPECTOMY (BREAST-CONSERVING NOTES)

I explained that her candidacy for breast conservation rests with Dr. [breast surgeon] and the oncology team, and that a lumpectomy is not a mastectomy: she will need radiation to the breast [per Dr. (radiation oncology)] and continued surveillance of both breasts with her breast surgeon, including the reduced contralateral breast. I explained that my role is partial breast reconstruction, filling the lumpectomy defect by rearranging her own breast tissue as a reduction, with the contralateral breast reduced in the same way at the same operation for symmetry, and that the treated breast is left slightly larger in anticipation of shrinkage from radiation. I explained that the reduction also reduces the weight of the

breasts and repositions the breast tissue and the nipple higher on the chest wall in order to relieve her symptoms of [neck, shoulder, and back pain, shoulder grooving, and intertrigo]. I explained that the nipple is carried on a [superomedial] dermoglandular pedicle that supplies its blood flow and sensation and provides the remaining breast tissue, and that the Wise-pattern incisions around the areola, down the lower pole, and along the inframammary fold allow the skin to be redraped over the remaining gland higher on the chest. I anticipate removing about [] g from the [left, including the lumpectomy specimen,] and [] g from the right (body surface area [], Schnur threshold [] g per breast). [The intended result was demonstrated on her own breasts / no modeling software was used.] I explained that the lumpectomy specimen will be oriented and the cavity marked with clips, and that if the final margins are positive, then re-excision is more difficult after tissue rearrangement and may require re-excision through the same incisions or a completion mastectomy, in which case the reconstruction plan would change to []. I explained that the contralateral reduction specimen is examined by pathology. [If genetics are pending: "I explained that the surgical plan will be finalized when the genetic result returns, because a pathogenic variant would change the recommendation toward mastectomy."] [If SN-N is long: "Given her sternal notch to nipple distance of () cm, I explained that the risk of nipple loss is higher than usual, that a free nipple graft may be needed with loss of sensation and pigmentation, and that if the nipple is lost then it would be reconstructed at a later stage, and I emphasized these risks because a wound in the treated breast could delay her radiation."] Patient verbalized understanding.

M19. FLAP OPTIONS CLOSED BY PRIOR SURGERY (ANY NOTE)

I explained that her prior [abdominoplasty / extensive liposuction] removes the abdomen as a donor site, so a DIEP flap is not possible, and that her prior [back lift / thoracic scars] crossing [the lateral inframammary fold] places local perforator flaps such as a LICAP or TDAP at higher risk of poor perfusion. [The remaining flap options, should she ever need a mastectomy, are ().] Patient verbalized understanding.

M20. REMOVAL OF RECONSTRUCTION IMPLANTS, WITH OR WITHOUT A NIPPLE LIFT (REVISION NOTES)

I explained the options after removal of her implants: removal with a flat closure; removal with excision of the excess skin and elevation of the nipple on the remaining mastectomy skin flap; removal with exchange for a smaller implant; removal with fat grafting at a later stage for contour; and no surgery. I explained that after mastectomy there is no breast tissue beneath the skin, so removal leaves a flat chest with a loose skin envelope and low nipples, and that elevation of the nipple depends entirely on the blood supply through the mastectomy skin flap, which [her transverse scars above the nipples / prior radiation to the () side] compromise. I explained that ICG angiography will be used during surgery to judge that blood supply, that if the nipple cannot be elevated safely, then the implants will be removed and the incisions closed flat so she can heal, with elevation of the nipples revisited at a later stage, and that if there is concern for loss of the nipple, then the nipple will be removed and replaced as a free nipple graft with loss of sensation and possible loss of pigmentation. I explained that partial nipple necrosis is possible even when the lift proceeds. I explained that the capsule [will be removed in full / will be removed where it is thickened or calcified / will be left in place] and sent to pathology, [that her implants are () and that textured implants carry a small risk of breast implant-associated anaplastic large cell lymphoma, so any fluid and the capsule will be sent for cytology and CD30]. I explained that radiated skin heals more slowly and that the lift on that side will be more conservative. Patient verbalized understanding and elected [].

Part 7. Sign-off consistency check

Before signing, confirm that the laterality, the mastectomy type, the nipple decision, the reconstruction type, and the staging are the same in the history, the exam, the assessment, and every paragraph of the plan; that every option set aside has a reason; that the adjuvant sentence names the oncologists and the basis for the radiation estimate; that the Caprini score is a number with a plan; that the risk sentence matches the chosen pathway; that no education or consent

text remains for a modality the patient is not receiving; and that the three visits (preoperative in person, within 30 days, 90-day photographs) carry dates. Confirm also that every consultant is named with the date of the report; that the anesthesia type, the admission plan, and the estimated duration are stated; that every order carries its indication; that the ABPS consent is described as signed, dated, and witnessed; and that the note is signed today. Any later addendum is dated and labeled as an addendum; nothing in a selected case is edited after the July 2027 selection notice without listing it on the EMR attestation.

Part 8. Case Log fields decided at this visit (tracker entry, not note text)

Fill the tracker row from the note before the patient leaves; the same facts print on the Case Log and on the case report title page.

1. **Anesthesia type:** general (the Log's choices are local only including nerve block, IV sedation, general, none).
2. **Admission status:** inpatient for any overnight stay, including 23-hour observation; outpatient only for a same-day discharge.
3. **Planned procedure description, in words, not CPT descriptors:** name the flap and say "microsurgical" or "free flap" when it applies; name the plane, mesh, and device for expanders and implants; list every site.
4. **Planned CPT codes with modifiers, as they will be billed** (verify with billing; the Log designates Major or Minor from these). Bilateral procedures are one code with -50, for example 19357-50. Common codes in this practice: 19357 tissue expander placement; 19340 immediate implant at mastectomy; 19342 delayed implant; 11970 expander to implant exchange; 11971 expander removal without implant; 19364 free flap breast reconstruction (DIEP, PAP, SGAP); 19361 latissimus flap; 19367 to 19369 pedicled TRAM; 15777 biologic mesh add-on; 15771 and 15772 fat grafting to the breast; 19380 revision of reconstructed breast; 19350 nipple reconstruction; 19370 and 19371 capsule procedures; 19318 reduction, 19316 mastopexy, and 19325 augmentation for contralateral symmetry; 15860 ICG perfusion assessment.
5. **Estimated duration, skin to skin, plastic surgery portion only:** [3 to 4 hours for expander or implant / 6 to 8 hours for bilateral DIEP].
6. **Anatomy and Category:** Breast; General Reconstructive (contralateral symmetry procedures under WHCRA are also General Reconstructive).
7. **Co-surgeon field:** not checked when the mastectomy is by a breast surgeon; checked only for a case with another plastic surgeon where I did the preoperative assessment, made the final decision, and provided all postoperative care, with my name on a separate operative report.
8. **Patient identifier:** the same number and at least two initials on every case for this patient at every facility.
9. **Stages expected inside July 1, 2026 to March 31, 2027:** []; only three per patient count toward the 50, all are entered.
10. **Intraoperative photograph plan, after incision and before closure, every site, every side:** expander or implant: the pocket with the mesh and device in place and the ICG still images; flap: the defect, the raised flap on its perforators, the flap inset before closure, and the donor site; nipple graft: the harvested graft and the recipient bed; office procedures: the wound before and after debridement.
11. **Imaging to keep for the radiology tab:** dated key images with reports (CT angiogram perforator images, MRI); mammography reports without images are sufficient.
12. **Research protocol:** [no / yes, with IRB approval and study consent on file].
13. **Office procedures later in this patient's course** (debridement, seroma aspiration, revision under local) are cases too: each gets a procedure note, photographs before and after, and a CPT code, and each is entered on the Log.